



Public Competition Assessment

12 May 2008

Lake Imaging & Western Medical Imaging - proposed merger

Introduction

1. On 19 March 2008, the Australian Competition and Consumer Commission (**ACCC**) announced its decision to not oppose the proposed merger between Lake Imaging (**Lake**) and Western Medical Imaging (**WMI**) (**proposed merger**). The ACCC was of the view that the proposed merger would be unlikely to have the effect of substantially lessening competition in contravention of section 50 of the *Trade Practices Act 1974* (**the Act**), in regional markets for the supply of:
 - i. diagnostic imaging services (excluding diagnostic imaging reporting services, nuclear medicine and MRI¹ imaging services) to public and private patients (excluding in-patients at public hospitals);
 - ii. diagnostic imaging reporting services (excluding diagnostic imaging services) to diagnostic imaging service providers; and
 - iii. nuclear medicine to public and private patients (excluding in-patients at public hospitals)
2. The ACCC formed its view on the basis of the information provided by the merger parties and information arising from its market inquiries. This Public Competition Assessment outlines the basis on which the ACCC reached its decision on the proposed merger, subject to confidentiality considerations.

Public Competition Assessment

3. To provide an enhanced level of transparency and procedural fairness in its decision making process, the ACCC issues a Public Competition Assessment for all transaction proposals where:
 - a merger is opposed;
 - a merger is subject to enforceable undertakings;
 - the merger parties seek such disclosure; or

¹ Magnetic resonance imaging.

- a merger is not opposed but raises important issues that the ACCC considers should be made public.
4. This Public Competition Assessment has been issued because Lake's proposed merger with WMI is considered to raise issues of interest to the public.
 5. By issuing Public Competition Assessments, the ACCC aims to provide the public with a better understanding of the ACCC's analysis of various markets and the associated merger and competition issues. It also alerts the public to the circumstances where the ACCC's assessment of the competition conditions in particular markets is changing, or likely to change, because of developments.
 6. Each Public Competition Assessment is specific to the particular transaction under review by the ACCC. While some transaction proposals may involve the same or related markets, it should not be assumed that the analysis and decision outlined in one Public Competition Assessment will be conclusive of the ACCC's view in respect of other transaction proposals, as each matter will be considered on its own merits.
 7. Many of the ACCC's decisions will involve consideration of both non-confidential and confidential information provided by the merger parties and market participants. In order to maintain the confidentiality of particular information, Public Competition Assessments do not contain any confidential information or its sources. While the ACCC aims to provide an appropriately detailed explanation of the basis for the ACCC decision, where this is not possible, maintaining confidentiality will be the ACCC's paramount concern, and accordingly a Public Competition Assessment may not definitively explain all issues and the ACCC's analysis of such issues.

The parties

Lake Imaging

8. Lake is a partnership comprised of radiologists and radiographer²/managers. Lake operates as a private diagnostic imaging services provider (excluding the supply of diagnostic imaging reporting services) to public and private patients from 16 locations throughout regional Victoria and north-west metropolitan Melbourne. Lake operates radiology practices in the Ballarat, Melton, Geelong and Daylesford regions of Victoria.
9. In 2006, Lake acquired the nuclear medicine and ultrasound components of the St John of God Health Care (SJGHC) hospital's diagnostic imaging services business in Ballarat. Lake has continued to provide these services to the SJGHC hospital since that acquisition.

² A professional trained to operate equipment, such as x-ray equipment, used to diagnose medical conditions. Radiographers are also known as imaging technicians.

Radsonic

10. Radsonic is a separate partnership comprised of the same radiologists that are in the Lake partnership (excluding the radiographer/managers in the Lake partnership).
11. Amongst other customers, Radsonic supplies diagnostic imaging reporting services³ to Lake and its competitor, Ballarat Health Services (**BHS**) under contract (the **BHS contract**).

Western Medical Imaging

12. WMI is a group of three corporate entities; Western Medical Imaging Pty Ltd, Victorian Medical Imaging Pty Ltd, and Eastern Medical Imaging Pty Ltd. Two of its radiologists are the current sole directors and shareholders of each of the WMI group of companies.
13. WMI operates as a private diagnostic imaging services provider (including the supply of diagnostic imaging reporting services) to public and private patients from 10 locations throughout regional Victoria and metropolitan Melbourne. WMI provides diagnostic imaging services in the Ballarat, Mount Waverley, Sunbury, Romsey and Kyneton regions and Broadmeadows in the north-west metropolitan Melbourne region. It also supplies diagnostic imaging reporting services under arrangements with numerous regional health services and other entities. These are located in Ballarat, outer metropolitan Melbourne, Maryborough, Stawell, West Wimmera, Ararat, Broadford, Yea, Wallan and Warragul.
14. In 2006, WMI established Ballarat Radiology, which was located across the road from the SJGHC hospital.
15. In 2007, WMI acquired the remaining components of the SJGHC hospital's diagnostic imaging services business in Ballarat that were not sold to Lake in 2006. WMI has continued to provide those services to the SJGHC hospital since the acquisition. Prior to the acquisition, WMI radiologists (directors/shareholders) were individually contracted to the SJGHC hospital.
16. In 2007, WMI ceased operating from across the road from the SJGHC hospital and now operates exclusively from the SJGHC hospital.

Other industry participants

Ballarat Health Services

17. BHS was established on 1 January 1997, bringing together three public health services; Queen Elizabeth Centre, Ballarat Base Hospital and Grampians Psychiatric Services.

³ Diagnostic imaging reporting services are the diagnosis and medical opinion of diagnostic images, which can only be provided by accredited radiologists.

18. BHS operates the Ballarat Base Hospital, which is the largest and principal referral hospital in the Ballarat region. BHS employs approximately 3,000 staff.
19. BHS provides diagnostic imaging services (excluding the supply of diagnostic imaging reporting services) to both public and private in-patients, and to public and private out-patients. BHS provides general diagnostic imaging services including ultrasound, CT⁴, angiography, fluoroscopy and dental services. While BHS employs its own radiographers and sonographers⁵ (both of which are imaging technicians), diagnostic imaging reporting services are provided under contract by Radsonic radiologists.

Primary Health Care Ltd

20. Primary Health Care Ltd (**Primary**) operates a medical centre in Ballarat (trading as the Eureka Medical Centre). The Eureka Medical Centre commenced operations in Ballarat on 24 April 2006. It primarily operates as a seven day general practitioner bulk billing clinic and incorporates a pathology collection centre and diagnostic imaging services including CT, ultrasound, x-ray and dental services. Primary's diagnostic imaging arm of the Eureka Medical Centre trades as Healthscan Specialist Imaging.
21. Healthscan Specialist Imaging employs a radiographer and a sonographer, with any shortfall covered by locum staff. Healthscan Specialist Imaging uses the services of a local radiologist (for high acuity procedures that require the presence of a radiologist) and uses teleradiology⁶ for most of its reporting needs. The local radiologist used is also an employee of WMI.

The proposed transaction

22. Lake and WMI will merge into one entity providing diagnostic imaging services, and is expected to acquire the supply of diagnostic imaging reporting services from Radsonic under contract.
23. The Radsonic partnership and WMI directors/shareholders will merge into a single group that will supply diagnostic imaging reporting services to:
 - the merged entity;
 - BHS; and
 - some regional health services contracts (previously supplied by WMI).

⁴ Computed tomography.

⁵ A professional trained to operate ultrasound equipment used to diagnose medical conditions. Sonographers are also known as imaging technicians.

⁶ Teleradiology involves the electronic transmission of diagnostic images, in digital form, from one location to a diagnostic imaging reporting service at another location. It can be used to provide diagnostic imaging reporting services where a radiologist does not need to be present for the provision of the diagnostic imaging service.

Industry background

Diagnostic imaging services

24. Diagnostic imaging services are the process of diagnosing conditions of the human body through the generation and interpretation of images using specialised equipment that are often referred to as modalities. There are two professional diagnostic imaging disciplines; diagnostic radiology and interventional radiology.
25. Modalities include, but may not be limited to, x-ray, fluoroscopy, ultrasound, mammography, CT scans, MRI, bone mineral densitometry and nuclear medicine.
26. In Australia, diagnostic imaging services are normally provided by:
 - i. public hospitals to public patients and private patients;
 - ii. private hospitals to private patients; and
 - iii. private clinics to public and private patients.
27. The provision of these services may take a number of forms. For example, a private or public hospital may:
 - i. subcontract the whole service to a private provider located onsite;
 - ii. subcontract some of the key inputs, such as the diagnostic imaging reporting services (discussed below); or
 - iii. self-supply the whole service.
28. The cost to patients for privately operated diagnostic imaging services is subsidised either wholly or partially by the Australian Government through Medicare rebates. The total cost of these services can also be met through co-payments made by patients, private health insurance and other Australian Government rebates for some procedures not included on the Medicare schedule.
29. General practitioners (GPs) and specialists make referrals to diagnostic imaging service providers. The choice by GPs and specialists as to which service provider to refer the patient is determined by a range of factors including quality, convenience, price, location, services available and clinical relationship with the reporting radiologist. Most doctors refer patients to a provider of choice, however, some do refer patients to a particular radiologist.
30. The demand for diagnostic imaging services is growing due to an expanding (and ageing) population, developing technologies and a greater focus on early detection (screening).

Diagnostic imaging reporting services

31. Diagnostic imaging services can also be split into two functional levels, diagnostic imaging, which involves the use of diagnostic imaging equipment to obtain images pertinent to providing a diagnosis, and diagnostic imaging reporting services, which are the interpretation and diagnosis of conditions based on those images.
32. The provision of diagnostic imaging reporting services involves the interpretation and analysis of imaging results to provide a diagnosis, and the provision of clinical reports. This may also include the analysis of patient history, or discussions with referring doctors, patients, or other radiologists.
33. Diagnostic imaging reporting services must be carried out by a radiologist, that is, a medical doctor with a suitable qualification in radiology. Some diagnostic imaging procedures require the presence of a radiologist during the imaging procedure (usually for interventional radiology procedures), while others do not require a radiologist's presence. Further, radiologists often consult with other specialists regarding their diagnosis of a diagnostic image, particularly for more complex or acute situations. For these reasons, hospitals require the ongoing physical presence of radiologists during business hours (and 'on-call' for after hours services) given the likelihood that patients could require the immediate services of a radiologist at any time, either for consultations, diagnosis or interventions, for example, in emergency situations.

High acuity and low acuity diagnostic imaging reporting services

34. The demand for diagnostic imaging reporting services from diagnostic imaging service providers that offer high acuity services differs to the demand for diagnostic imaging reporting services from diagnostic imaging service providers that offer low acuity services for the following reasons:
 - i. diagnostic imaging service providers with high acuity services mostly provide services to hospitals with acute needs such as emergency departments. Such high acuity services demand that radiologists are physically available to provide immediate analysis of diagnostic imaging results, view a patient's medical history, provide immediate consultation with other specialists, and conduct interventional radiology. Contractual arrangements for high acuity providers also require radiologists to make themselves physically available for 'on-call' situations after normal business hours;
 - ii. diagnostic imaging service providers with low acuity services can more readily rely on teleradiology for its supply of diagnostic imaging reporting services and may schedule any small volume of high acuity needs, such as interventional radiology, at certain times to the convenience of the radiologist; and
 - iii. teleradiology is only partially available for the provision of high acuity diagnostic imaging reporting services due to the clinical and regulatory need for radiologist collocation with the patient.

35. Whilst both low acuity and high acuity diagnostic imaging services have a regulatory requirement for supervision of interventional radiology, the nature of high acuity providers—seen in acute hospitals and/or specialised private clinics—demand a greater degree, volume and level of interventional radiology and thus a greater need for the permanent presence of a radiologist.

The Ballarat region

36. Diagnostic imaging services are provided in the Ballarat region by Lake, WMI, Primary (trading as Healthscan Specialist Imaging) and BHS.
37. Lake operates from three premises in the region, including its premises at the SJGHC hospital. Lake is a vertically integrated entity in the sense that it self supplies its own diagnostic imaging reporting services (through Radsonic).
38. WMI operates from a single location at the SJGHC hospital. WMI operates next to, and competes with, Lake at the hospital. There is also some overlap of the floor space between the two. WMI is also vertically integrated by self supplying its own diagnostic imaging reporting services.
39. Primary operates a partially integrated diagnostic imaging service, collocated with its own GPs within the Eureka Medical Centre. Primary is located away from the medical district in Ballarat where its competitors are based. Primary is a relatively new entrant to the market and does not receive many referrals outside of its own GP network. Primary has an agreement with a WMI radiologist to provide some short sessions per week (enough for interventional work). Primary's remaining diagnostic imaging reporting services are conducted via teleradiology with its own radiologists based in Melbourne.
40. BHS operates from the Ballarat Base Hospital, which is across the road from the SJGHC hospital. In addition to providing services to public and private in-patients and public out-patients at the Ballarat Base Hospital, BHS competes with Lake and WMI for private out-patients. Unlike Lake, WMI and Primary, BHS is not vertically integrated and receives its diagnostic imaging reporting services from Radsonic under contract.

Timing

41. The following table outlines the timeline of key events in this matter.

Date	Event
11-Jan-2008	ACCC commenced review under the Merger Review Process Guidelines.
01-Feb-2008	Closing date for submissions from interested parties.
27-Feb-2008	ACCC published a Statement of Issues outlining preliminary competition concerns.
07-Mar-2008	Closing date for submissions relating to the Statement of Issues.
19-Mar-2008	ACCC announced it would not oppose the proposed merger.

Areas of overlap

42. The ACCC considers that the main area of overlap between Lake and WMI's businesses is in the supply of diagnostic imaging and diagnostic imaging reporting services in the Ballarat region.
43. The ACCC considered that there was no or negligible overlap in the supply of diagnostic imaging and diagnostic imaging reporting services in the following regions: Melton; Geelong; Daylesford; north-west and outer metropolitan Melbourne; Mount Waverley; Sunbury; Romsey; Kyneton; Maryborough; Stawell; West Wimmera; Ararat; Broadford; Yea; Wallan; and Warragul. As such these regions were not considered further.
44. The ACCC considered that there was no or negligible overlap in the supply of nuclear medicine in any regional markets. As such this market was not considered further (see para 51).

Market inquiries

45. The ACCC conducted market inquiries with a range of industry participants, including competitors, potential competitors, referring doctors, input suppliers, industry bodies and other interested parties. Submissions were sought in relation to the substantive competition issues.

Statement of issues

46. After an initial round of market inquiries, the ACCC published a Statement of Issues on 27 February 2008 identifying a number of preliminary competition concerns with the proposed merger, including the following possible unilateral effects:
 - i. Horizontal
 - a. The potential for the proposed merger to substantially lessen competition in the supply of diagnostic imaging services.
 - b. The potential for the proposed merger to substantially lessen competition in the supply of diagnostic imaging reporting services.
 - ii. Vertical
 - a. The potential for the merged entity to have the ability and incentive to foreclose downstream competition in the supply of diagnostic imaging services.

Future with/without test

47. In assessing a merger pursuant to section 50 of the Act, the ACCC considers the effects of the transaction by comparing the likely competitive environment post-merger if the transaction proceeds (the “with” position) to the likely competitive environment post-merger if the transaction does not proceed (the “without” position).

Market definition

Product market

48. On the supply-side, the ACCC took into account the following facts: diagnostic imaging reporting services are only supplied by Radsonic and WMI; diagnostic imaging reporting services require distinct skill sets and formal qualifications over and above those required by imaging technicians; and diagnostic imaging reporting services, depending on their nature can be, and sometimes are, performed from a remote location to the patient by independent parties.
49. On the demand-side, the ACCC took into account that diagnostic imaging reporting services are a vital input to the supply of diagnostic imaging services and Lake, BHS and Primary did not directly provide diagnostic imaging reporting services requiring the supply of these vital services.
50. Based on this, the ACCC concluded that the market for supply of diagnostic imaging services is a separate product market to the market for the supply of diagnostic imaging reporting services. This finding was not contested during the ACCC’s market inquiries.
51. The ACCC considered separate product markets for the supply of nuclear medicine and MRI diagnostic imaging because the procedures are not as common and are more complex than other forms of diagnostic imaging. For example, radiologists require additional qualifications to supply nuclear medicine and the Australian Government limits the supply of Medicare-eligible MRI services through licensing arrangements.

High and low acuity services

52. The ACCC was also of the view that the markets for the supply of diagnostic imaging services and diagnostic imaging reporting services had two distinct market segments; low acuity services and high acuity services (discussed at para 34-35). Whilst both low acuity and high acuity services require radiologist consideration, interpretation and diagnosis, distinct differences include:
- i. high acuity services require the physical presence of the radiologist to conduct interventional radiology (for example dye treatments)⁷, for immediate analysis of results and immediate consultation with other

⁷Part 2, *Health Insurance Act 1973* (Cth).

specialists (for example hospital trauma cases), and availability on-call for emergency matters; and

- ii. low acuity services are those diagnostic imaging services conducted within normal business hours with less urgent result turnaround needed and often not requiring interventional radiology or the physical presence of the radiologist. Low acuity services can be supplied using teleradiology.
53. High acuity services therefore distinguish between, diagnostic imaging services and diagnostic imaging reporting services provided to hospitals with emergency and/or intensive care units from other diagnostic imaging clinics (either within a hospital or the community).

Geographic market

54. The ACCC considered that the geographic boundaries of the market are difficult to precisely define. Basic imaging services (for example x-rays) tend to be offered by more providers, and accordingly the markets tend to be smaller. More complex procedures (for example fluoroscopy), or providers with high-end equipment (for example multi-slice CT), which are often less common draw from a wider geographic area. The ACCC considered that the Ballarat region was consistent with the 'catchment area' of patients that seek the supply of diagnostic imaging services and diagnostic imaging reporting services.

Conclusion

55. The ACCC concluded that the relevant markets include:
- i. the market for the supply of diagnostic imaging services (excluding diagnostic imaging reporting services, nuclear medicine and MRI imaging services) to public and private patients (excluding in-patients at public hospitals) in the Ballarat region; and
 - ii. the market for the supply of diagnostic imaging reporting services (excluding diagnostic imaging services) to diagnostic imaging service providers in the Ballarat region.

Possible competitive harm

Unilateral effects: horizontal – supply of diagnostic imaging services

56. The ACCC considered that price competition within the supply of diagnostic imaging services related to the out-of-pocket expenses for patients. That is, the size of the co-payment that is needed to be paid by a patient for a service.

Rivalry between Lake & WMI

57. Market inquiries indicated that WMI has not been a strong competitor in the Ballarat region and has only provided a limited constraint on Lake's operations and would likely continue to do so absent the proposed merger.

Competing diagnostic imaging services

58. Market inquiries indicated that BHS could effectively compete on service levels, convenience, imaging technology and price. BHS currently provides lower prices than Lake and WMI and would likely continue to do so post-merger.
59. It was also found that Primary could effectively compete on price whilst maintaining similar levels of service and would likely continue to do so post-merger. Primary is the only competitor that bulk bills all of its diagnostic imaging patients. Primary does not currently fully utilise its equipment and staff which would enable it to cater for an increase in demand.
60. Accordingly, the ACCC considered that BHS and Primary would be likely to provide an effective competitive constraint if the merged firm attempted a significant and sustainable increase in price post-merger.
61. The ability of competing diagnostic imaging services providers to secure access to the supply of the vital input of diagnostic imaging reporting services is discussed from para 77 to 83.

New diagnostic imaging services

62. Market inquiries verified that barriers to entry exist in:
- i. attracting staff willing to perform 'on-call' work;
 - ii. attracting imaging technicians to regional towns;
 - iii. incumbency advantages, such as securing a referral network from doctors;
 - iv. acquiring premises within the Ballarat hospitals or medical precinct; and
 - v. securing radiologists to supply diagnostic imaging reporting services;
63. The ACCC noted that each of the above factors hindered the likelihood and ability for a new entrant to provide an important source of competitive constraint. Of particular concern was the difficulty in securing the supply of diagnostic imaging reporting services due to a general shortage of radiologists around Australia, particularly regional Australia. Equally strong was the difficulty in securing a referral stream of patients, due to established relationships and levels of expectations between doctors and incumbents. Incumbency barriers to the diagnostic imaging services market would be overcome if a new entrant was

successful in obtaining the BHS contract (discussed in para 70 to 73). Market inquiries showed that Primary had not achieved market share growth, nor had it attracted its own permanent Ballarat based radiologist since it entered the market 2006, despite its competitive pricing. The ACCC considered that effective new entry, with sufficient scope, was not likely for a substantial period of time.

64. Market inquiries indicated that lower barriers to entry exist for low acuity diagnostic imaging services compared to high acuity diagnostic imaging services. The potential exists for new entrants to offer low acuity diagnostic imaging services by employing locally based or commuting employees, operating normal business hours, and purchasing diagnostic imaging reporting services using teleradiology technology if necessary.
65. Accordingly, the ACCC considered that barriers to entry for new diagnostic imaging services providers are moderate for low acuity diagnostic imaging services and high for high acuity services, however, noting the advantages of the BHS contract to a new entrant.

Conclusion

66. For the purposes of assessing the proposed merger, the ACCC considers that competition from BHS and Primary would provide an effective competitive constraint and noted that WMI would provide a limited competitive constraint with or without the proposed merger. The ACCC therefore took the view that the proposed merger would be unlikely to substantially lessen competition for the purposes of section 50 of the Act through horizontal effects in the supply of diagnostic imaging services (excluding diagnostic imaging reporting services, and nuclear medicine and MRI imaging services) to public and private patients (excluding in-patients at public hospitals) in the Ballarat region.

Unilateral effects: horizontal – supply of diagnostic imaging reporting services

67. The proposed merger will effectively result in a single group of radiologists supplying all high acuity diagnostic imaging reporting services and the majority of low acuity diagnostic imaging reporting services to the Ballarat region. The ACCC considered the potential impact of a high concentration of radiologists with an economic interest in the merged firm on BHS and Primary, given their current reliance on the merger parties for Ballarat based radiologists to provide diagnostic imaging reporting services, and the competitive constraints exhibited by potential new entrants.

Rivalry between Lake & WMI

68. Market inquiries indicated that WMI has not been a strong competitor in the Ballarat region and has only provided a limited constraint on Radsonic's operations and would likely continue to do so absent the proposed merger.

New diagnostic imaging reporting services

69. Market inquiries indicated that for high acuity diagnostic imaging services, radiologists need to be on-site during normal business hours and/or immediately available 'on-call' for after hours services. Further, diagnostic imaging services in hospitals tended to be very busy and have high volumes of patients. Therefore a critical mass of radiologists (who are in short supply) is needed to service a high acuity contract at a public hospital, such as the BHS contract, and only Radsonic or one of Australia's major diagnostic imaging companies is likely to have this ability.
70. In addition, market inquiries indicated that some of Australia's major diagnostic imaging companies would bid for the BHS contract if or when it came up for renewal, provided the incentives were suitable. The largest barrier would be the ability to attract the critical mass of radiologists needed to service such a contract. Market inquiries suggested that these large diagnostic imaging providers were in the best position to overcome this barrier because they already have a large pool of employed radiologists to which they could provide incentives to move to Ballarat.
71. The price and service level of the current BHS contract will continue until a new contract is negotiated. Therefore, the competitive effects of the proposed merger, in relation to any new contract with BHS will not be evident until such time it is re-negotiated (if at all). Market inquiries indicated that WMI has not been a strong competitor in the Ballarat region and has only provided a limited constraint on Lake's operations and would likely continue to do so absent the proposed merger.
72. Market inquiries indicated that Primary's ability to secure high acuity diagnostic imaging reporting services would not substantially alter with or without the proposed merger.
73. Accordingly, the ACCC considers that barriers to entry for organic new suppliers of high acuity diagnostic imaging reporting services are high, however, key barriers to entry could be overcome if a new entrant secured a contract with BHS.
74. Market inquiries showed that teleradiology technology lowered barriers to entry for low acuity diagnostic imaging reporting services as there is no clinical need for the actual presence of a radiologist for the vast majority of work (although desirable). This was supported by Primary's use of teleradiology for its low acuity services.
75. Accordingly, the ACCC considers that moderate barriers to entry exist in the provision of low acuity diagnostic imaging reporting services for the purposes of assessing the proposed merger.

Conclusion

76. For the purposes of assessing the proposed merger, the ACCC considers that competition for the BHS contract is likely to provide an effective competitive constraint and noted that WMI would provide a limited competitive constraint with or without the proposed merger. The ACCC therefore took the view that the proposed merger was unlikely to substantially lessen competition for the purposes of section 50 of the Act through horizontal effects in the supply of diagnostic imaging reporting services (excluding diagnostic imaging services) to diagnostic imaging service providers in the Ballarat region.

Unilateral effects: vertical – foreclosure of downstream competition

77. The proposed merger will effectively result in a single group of radiologists supplying all high acuity and the majority of low acuity diagnostic imaging reporting services to the Ballarat region. Diagnostic imaging reporting services are a vital input in supply of diagnostic imaging services. The ACCC considered the ability and incentive for Radsonic to use its position as the single provider to foreclose (or otherwise discriminate) its supply of diagnostic imaging reporting services to undermine competition and favour the Lake diagnostic imaging services business in which it has an economic interest.
78. For example, Radsonic could attempt to restrict inputs or raise costs for rivals of Lake's diagnostic imaging services business to either:
- i. restrict the supply of diagnostic imaging reporting services to BHS and Primary;
 - ii. restrict the level of service it provides to BHS and Primary; or
 - iii. raise the price of diagnostic imaging reporting services it supplies to BHS and Primary

Ability, incentive and likely effect of foreclosure

79. Market inquiries indicated that, despite any incentives, Radsonic did not have the ability to foreclose BHS because of contractual obligations protecting the supply, service level and price for diagnostic imaging reporting services. The ACCC considers that foreclosure would therefore be unlikely for the duration of the BHS contract.
80. The ACCC considers that on expiration of the BHS contract Radsonic would have the ability to foreclose BHS but would not have the incentive to foreclose BHS. Market inquiries indicated that it was likely that Radsonic would effectively compete with potential new entrants (discussed in para 70 to 73) to secure a future BHS contract and would therefore have no incentive to foreclose.

81. The ACCC considers that the threat of new entrants for a future BHS contract would ensure that Radsonic does not have the incentive to foreclose BHS if it was successful at obtaining a new contract with BHS. Market inquiries also suggested that BHS has the option and capacity to alter the structure of how it acquired its future diagnostic imaging reporting services to its favour.
82. Market inquiries indicated that Radsonic would have the ability to foreclose Primary's access to a Ballarat based radiologist for high acuity diagnostic imaging reporting services. However, the ability of Primary to maintain access to a radiologist was not substantially different with or without the proposed merger.

Conclusion

83. Although Radsonic will be a single service provider in the upstream market for supply of diagnostic imaging reporting services, in particular to high acuity diagnostic imaging service providers, the ACCC took the view that the proposed merger did not substantially alter any ability or incentive for Radsonic to foreclose BHS or Primary and therefore was unlikely to substantially lessen competition for the purposes of section 50 of the Act in the supply diagnostic imaging services and diagnostic imaging reporting services in the Ballarat region.

Conclusion

84. On the basis of the above, the ACCC formed the view that the proposed merger would be unlikely to substantially lessen competition in the relevant market for the supply of diagnostic imaging services or diagnostic imaging reporting services in the Ballarat region in contravention of section 50 of the Act.